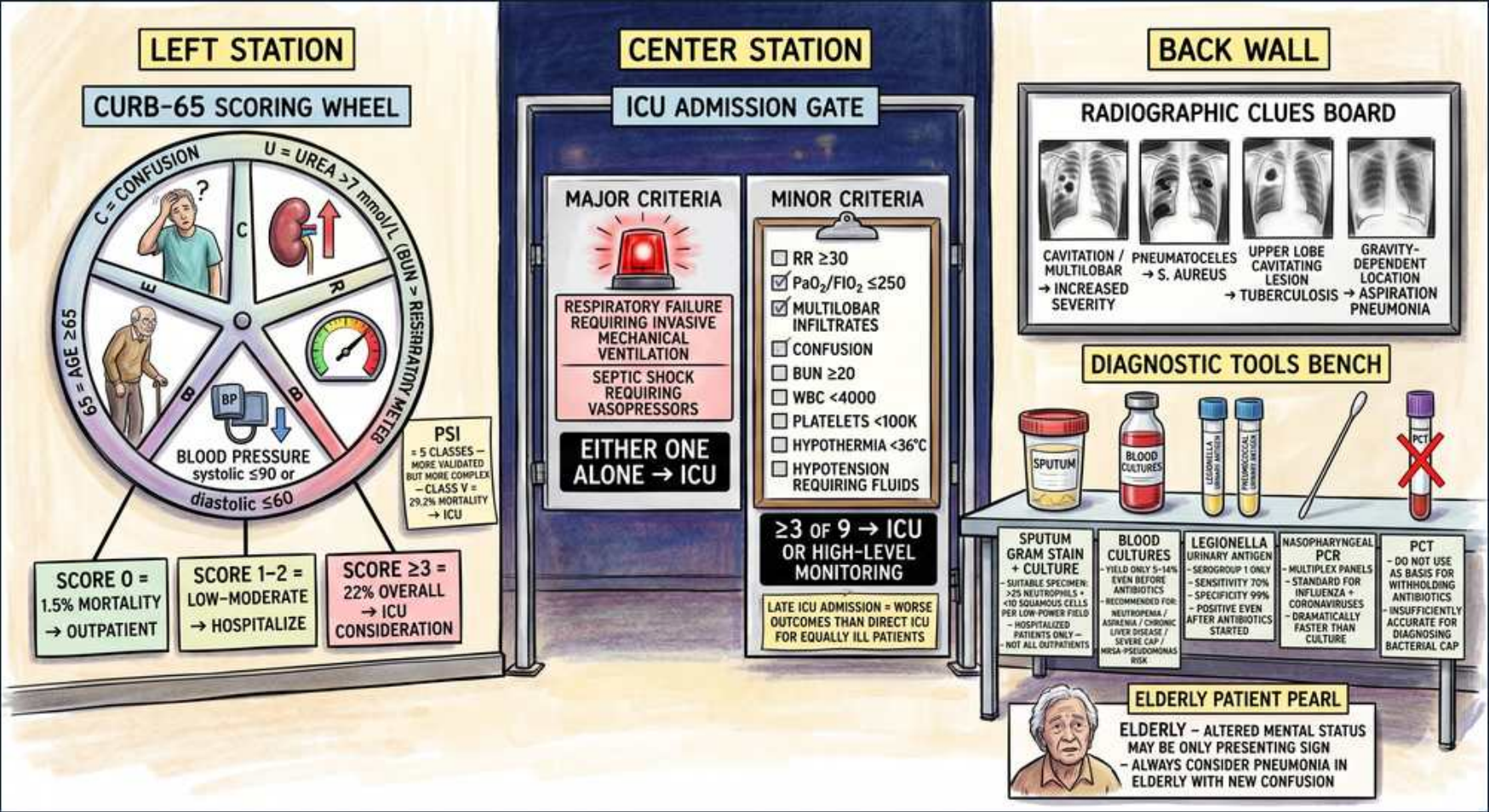


Pneumonia Severity: CURB-65, ICU Criteria & Diagnostics



1. CURB-65 wheel

WHAT YOU SEE

Left station scoring wheel

WHAT IT ENCODES

CURB-65 awards 1 point each for Confusion, Urea >7 mmol/L (BUN >19), Respiratory rate ≥30, Blood pressure <90/<=60, and age ≥65. It triages site of care for CAP.

BOARD TRAP

CURB-65 uses urea/BUN, not creatinine — don't confuse the renal marker; confusion here is new disorientation, not chronic dementia.

2. Confusion

WHAT YOU SEE

C segment, confused man

WHAT IT ENCODES

New-onset confusion/disorientation scores 1 point and independently predicts mortality. In the elderly it may be the only presenting sign of pneumonia.

BOARD TRAP

In an elderly patient new confusion alone warrants a pneumonia workup — don't anchor on a UTI or dementia before ruling out chest infection.

3. Urea / BUN >7

WHAT YOU SEE

U segment, kidney

WHAT IT ENCODES

Urea >7 mmol/L (BUN >19 mg/dL) scores 1 point, reflecting dehydration/renal hypoperfusion and worse prognosis.

BOARD TRAP

This is urea/BUN, not creatinine — a normal creatinine does not exclude the point.

4. RR ≥30

WHAT YOU SEE

R segment, respiratory rate

WHAT IT ENCODES

Respiratory rate ≥30/min scores 1 point and is also an ICU minor criterion. A key marker of respiratory compromise.

BOARD TRAP

Tachypnea is one of the most sensitive severity signs — don't dismiss an RR of 32 just because oxygen saturation looks acceptable.

5. BP <90 / ≤60

WHAT YOU SEE

B segment, blood-pressure cuff

WHAT IT ENCODES

Systolic <90 or diastolic ≤60 mmHg scores 1 point. Hypotension flags sepsis physiology and need for fluids/vasopressors.

BOARD TRAP

Even isolated diastolic ≤60 counts — don't require frank shock to award the BP point.

6. Score interpretation

WHAT YOU SEE

Score boxes 0 / 1-2 / ≥3

WHAT IT ENCODES

Score 0 = ~1.5% mortality, outpatient; 1-2 = low-moderate, consider admission; ≥3 = ~22% mortality, hospitalize and consider ICU.

BOARD TRAP

CURB-65 guides disposition, not antibiotic choice — and a score of 2 with social risk may still warrant admission despite the low-risk label.

7. PSI alternative

WHAT YOU SEE

PSI gauge near wheel

WHAT IT ENCODES

The Pneumonia Severity Index (PSI/PORT) is the more sensitive but complex tool; PSI class IV-V (>90 points) means ~25% mortality and inpatient care. CURB-65 is the simpler bedside alternative.

BOARD TRAP

PSI is better validated for identifying LOW-risk outpatients; CURB-65 is simpler but less granular — they are not interchangeable scores.

8. ICU major criteria

WHAT YOU SEE

Center gate, mechanical ventilation / septic shock

WHAT IT ENCODES

Either ONE major criterion mandates ICU: invasive mechanical ventilation, or septic shock requiring vasopressors.

BOARD TRAP

A single major criterion alone sends the patient to ICU — you do NOT need to also meet minor criteria when a major one is present.

9. ICU minor criteria

WHAT YOU SEE

Center gate, list of minors

WHAT IT ENCODES

Minor criteria include $RR \geq 30$, $PaO_2/FiO_2 \leq 250$, multilobar infiltrates, confusion, $BUN \geq 20$, $WBC < 4000$, platelets $< 100k$, hypothermia $< 36C$, and hypotension needing fluids. Need ≥ 3 for ICU.

BOARD TRAP

It takes ≥ 3 minor criteria (not 1-2) to justify ICU admission in the absence of a major criterion.

10. Cavitation/multilobar CXR

WHAT YOU SEE

Back wall radiograph board

WHAT IT ENCODES

Cavitation suggests anaerobes, *S. aureus*, *Klebsiella*, TB, or fungi; multilobar infiltrates indicate severity and an ICU minor criterion.

BOARD TRAP

Cavitary upper-lobe disease in the right host is TB/anaerobe/MRSA — don't default to ordinary pneumococcus.

11. Upper-lobe cavitation

WHAT YOU SEE

S. aureus / TB labeled film

WHAT IT ENCODES

Upper-lobe cavitation points to *S. aureus* or reactivation TB depending on host and tempo. Apical disease with night sweats and weight loss favors TB.

BOARD TRAP

Subacute upper-lobe cavitation with constitutional symptoms = TB; acute necrotizing cavitation post-flu = *S. aureus* — tempo distinguishes them.

12. Sputum gram stain/culture

WHAT YOU SEE

Diagnostic bench, sputum tube

WHAT IT ENCODES

A good sputum sample (> 25 PMNs, < 10 epithelial cells per LPF) guides organism-directed therapy in severe CAP. Most useful before antibiotics in hospitalized/ICU patients.

BOARD TRAP

A specimen with many epithelial cells is oropharyngeal contamination — don't treat based on a poor-quality sputum culture.

13. Legionella urinary antigen

WHAT YOU SEE

Bench, Legionella test

WHAT IT ENCODES

Urinary antigen detects Legionella serogroup 1 rapidly and stays positive after antibiotics. Order in severe CAP, ICU patients, and travel/outbreak exposure along with pneumococcal urinary antigen.

BOARD TRAP

A negative Legionella urinary antigen does not exclude non-serogroup-1 strains — use culture on BCYE if suspicion is high.

14. Procalcitonin

WHAT YOU SEE

PCT vial, bench right

WHAT IT ENCODES

Procalcitonin rises in bacterial infection and can support stopping antibiotics, but should NOT be used to withhold initial therapy in suspected CAP.

BOARD TRAP

Never delay empiric antibiotics for a low procalcitonin in clinically diagnosed pneumonia — PCT guides de-escalation, not the decision to start.

15. Elderly pearl

WHAT YOU SEE

Bottom, elderly patient face

WHAT IT ENCODES

In the elderly, altered mental status may be the only sign of pneumonia; always consider chest infection in a senior with new confusion even without fever or cough.

BOARD TRAP

Absence of fever/leukocytosis does not exclude pneumonia in the elderly — afebrile presentations are common and dangerous.
